

AR Medical billing training

Provider – AS A Doctor

Health Insurance –Payer - health insurance company

Patient – take treatment by provider

- 1) **SSN** - Social Security Number (9 Digits Numeric Number)/ AS A Adhar card
- 2) **NPI** - National Provider Identification (**10 Digits**) – AS A license number
- 3) **TIN** – Tax Identification Number (9 Digits) - AS A GST number
- 4) **CPT** - Current Procedural Terminology (5 Digit Numeric/ Alphanumeric) it is use for treatment
- 5) **ICD -10 (DX CODE)**- International Classification of Diseases (3 to 7 Digit Numeric/ Alphanumeric) it is use for **Diseases code.**
- 6) **COBRA** - Consolidated Omnibus Budget Reconciliation Act
when a Person left the organization any resin than insurance heath befits coverage continue for 18 months.
- 7) **HIPAA** - Health Insurance Portability and Accountability Act (1996, 21 August)
HIPPA is PROTECT patient health information.

POS (place of services) which location take the treatment by provider 11-12-21-22-23-

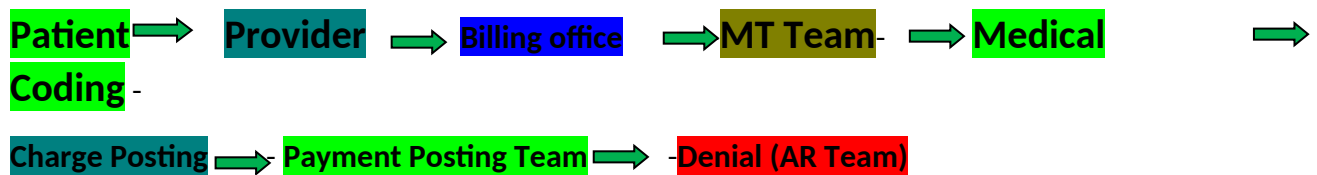
DOS (Date of services) which date take the treatment by provider

CMS1500 FORM USED FOR physician Billing, there are 33 box

UB04 FORM used for hospital Billing there 81 box

What is RCM (**Revenue Cycle Management**): Revenue Cycle Management (RCM) refers to the process of identifying, collecting and managing the practice's revenue from payers based on the services provided.

Flow chart of RCM Revenue Cycle Management



Q-Which insurance company / payer working in previous company

I worked with multiple insurance company like is ---

US Goverment Insurance	US Commercia Ins./Private
Medicare	BCBS BLUE CROSS BLUE SHLED Largest Commercial Insurance Company in US
Medicaid	Aetna
Tricare	Cigna
	Humana
	UHC
	Worker compensation (WC)

Billed Amount -Decided by provider as per fees schedule

Allowed amount -Decided by insurance company as per fees schedule called is Allowed amount

Paid amount = provider has received payment from primary insurance + secondary insurance)+Patient pay.

COB: Coordination of Benefits): - When a patient is having more than one insurance that is called COB. and it is decided by patient that which insurance pay as primary and which will pay as secondary.

PCP: Primary Care Physician): PCP is equivalent to the family doctor which refers the patient to specialist Provider With referral number.

What is referral number. Referral number provide by PCP. When patient 1st Visit to PCP then PCP refer to specialist provider with referral number.

Participating Provider (In – Network Provider): those providers having the contract with the insurance company. called Participate provider.

Non – Participating Provider (Out of network provider): Those providers who are not having the contract with the insurance company is called non-Participate provider

TFL: Timing Filing Limit: This is a time duration which is given by insurance company in which provider have to submit the claim to the insurance company. This is calculated from **Date of service.**

Medicare insurance 365 days, Medicaid 180 days. BCBS and Humana, Atena 120 days

Capitation This is a fix amount which insurance company has to pay to the provider on monthly basis. **PER MONTH PER MEMBER**

Authorization or Pre -Authorization: It is a kind of number which is taken by the provider from insurance company for **Some Expensive Treatment.**

EOB: Explanation of Benefits): - This is a document that is sent by insurance company to the provider which contains all the payment posting and Denial detail. **LIKE IS**

Billed amount, allowed amount, paid amount, copay, deduction, co-insurance CPT DOS adjustment amount (CO45, CO253). Patient detail and insurance company and Denial Detail, etc.

ERA: Electronic Remittance Advice): It is digital form of the EOB and it is use for auto posting.

Q-1 **What is patient reasonability- (PR)** Patient is paid to the provider like is , PR1,PR2,PR3

Q2- **What is out of pocket expensive** Patient pay by on pocket that is out of pocket. Like is PR1,PR2,PR3 AND BALANCE BILL

Patient Responsibility – Those are responsibility pay by patient to provider

Deductible (PR1): It is a fixed amount which is paid by patient or secondary ins to the provider on annual basis only then after insurance company start giving Benefit.

Co-insurance (PR2): It is specified (%) percentage of **Allowed amount** which is paid by patient or secondary Ins to the **provider.** -20/80

Copay (PR3): It is a small dollar amount which is paid by the patient to provider on every visit. It is a just like doctor fee

Re-fund: When an insurance company make access or incorrect payment, they take back that amount is called refund.

Recoupment: When an insurance company **process incorrectly EOB**, then they take back the **entire payment**, is called the recoupment

ABN: Advance Beneficiary Notice: - This is a form which is signed by patient, in this form mentioned that if **Medicare Denied making payment** than patient will be responsible for making payment to the provider

Hospice - The care which is given by Medicare when patient is suffering from **terminal illness** and will not survive for more than 5-6months.

There are two **Modifier used -GW AND GV**

AS PER , **POS -34**

HMO - Health Maintenance Organization	PPO- Preferred Provider Organization
1-HMO plan work only participate provider 2-In HMO plan REQUIRED - PCP REFERRAL NUMBER 3-Patient fist go to PCP Than PCP refer to specialist provider with referral number	1- In PPO plan work on participating provider As well as non-participating providers. In PPO Plan patient can directly go to the specialist Provider without a PCP referral.

Medicare Insurance: It is **Nation government** insurance and to get the Medicare insurance there are three eligibility criteria.

1st 65 years old or above.

2nd suffering from (ESRD) end stage renal disease. **KIDANI FAILER**

3rd Permanent disability.

Medicare Id **11** digit alphanumeric

Medicaid insurance: It is a **state government** insurance for **low-income group**. Medicaid is always last payer. **PATEINT NEVER PAY ANY AMOUNT**

Medicare Crossover Once Medicare has paid its portion it transfers the remaining balance to secondary insurance.

Capitation: This is a fix amount which insurance **company has to pay to the provider** on the basis of **per member per month**

CO253 (Sequestration adjustment): It is a 2% of **Medicare** payment reduction and it kinds of adjustment.

CO45- Contractual adjustment

How many parts of Medicare.

- **Medicare Part A** for **Hospital billing**. In patient
- **Medicare Part B:** for **Physician Billing**. Out patient
- **Part C:** **combination A and B**
- **Part D:** **drugs**

The Medicare Part B deductible for 2025 is \$257.

Medicare Crossover Once Medicare has paid its portion it transfers the remaining balance to secondary insurance

Medicaid insurance: It is a **state government** insurance for low-income group. Medicaid is always last payer. **PATEINT NEVER PAY ANY AMOUNT**

1st MEDICATE and 2nd ATENA = 1ST ATENA REST OF AMOUNT PAY BY MEDICATE

What is a spend down.

If a consumer has an income that exceeds the Medicaid standard, then consumer can use medical expenses that will reduce his income to the Medicaid need standard.

Trycare insurance -It is a government insurance for uniform Employee. like army

Worker Compensation ins.(WC) It is state government insurance which cover **work related injury, illness, or death**. WC is always Primary payer.

Important * **date of injury***

Two insurance . 1st Atena and 2nd WC . PRIMARY BILLING WC

Clearinghouse

Clearing house is an office which receives the claims from Provider in Electronic format,
clearing house check any missing info on the claims and if everything is correct then clearing house transfer the claim to payer in electronic format.

NCD - **National coverage determination** - is a general outline of coverage which is applicable regardless to which MAC (Medicare Administrative Contractor)

LCDs (**Local Coverage Determinations**) are specific to a Medicare Administrative Contractor (MAC).

Ageing claim complete

Place of Service: which location take services by provider called the **Palace of services (POS)**

11 - Office Visit

12 - Home Visit

21 - In Patient more than 24 hours

22 - Out Patient Less than 24 hour

23 - Emergency

13 - Assisted Living

31 - Nursing Facility

32 - Skilled Nursing Facility

Modifiers: Modifiers are two-digit alpha numeric code and enhance value of CPT With out changing it.

21 - When provider expends extra time with patient

24 - Post operative period. - **Unrelated services**

25 - FOR E&M. When provider perform procedure with E&M.

26 - (Professionals Services) When patient gets services outside. Then facility not available in hospital

50 - (Bilateral Procedure -Service performed by same operative session). When the patient the both side Organ treatment

51 - Multiple Procedures other than E&M

52 - Reduce Services

59 - (Distinct Procedure) Separate Services from other provider on same day

76 - Repeat procedure by same Physician SAME DAY

77 - Repeat procedure by other Physician ON SAME DAY

RT - Right side of Organ.

LT - Left side of Organ.

TC - Technical Components

1. Professional billing (Physician billing) claim form CMS (center for Medicaid and Medicare services) -1500 (33) boxes in claim form.

22- resubmission code = 6*7*8 (6for fresh claim,7for corrected,8-avoid

23- AUTHORIZATION

24- POS.DOS

25- TAX ID

31- PROVIDER SIGNED

32- facility location

33- PROVIDER ADDRESS

-Which specialty / practice work in previous company

Anesthesiology start from 0 - 00100 - 01999, 99100 - 99140

Surgery - start from 1 - 10040 - 69990 POS 21

Radiology - start from 7- 70010 - 79999 - POS 22 . MODIFIER
26

Pathology and Laboratory start from 8 - 80048 - 89399 POS 22

Medicine - start from 9

DENAIL

Work on mostly Top 5 DENIAL NAME.

1. NON-COVERED SERVICES (CO 96 AND PR 96)

2. INCLUSIVE/ BUNDLED ---

3. GLOBAL

4. NO AUTHORIZATION

5. TIMELY FILING LIMIT EXPIRED

6. DUPLICATE

7. CLAIM IS PAID

8. CLAIM NOT ON FILE.

9. COORDINATION OF BENEFIT .

10. MAXIMUM BENEFIT MET OR REACHED

11. CAPITATION

1- NON-COVERED SERVICES (CO96 AND PR96)

When the services are not **covered under the patient's plan** or **Provider specialty**, we will get such kind of denials.

We will call to insurance and ask **Denial Date** and **Claim Number**.

- We will verify **whether** service is not covered under the **patient plan** or **provider specialty** and we will take the reason for the same.
- We will take **call Reference**

1- If Service is **not covered** under the **Doctor Specialty** we will work as **per client protocol**.

2- If service is **not covered** under **patient plan** We will bill the claim to any other **active Insurance** otherwise bill to **patient**.

2-Maximum benefit met/reached (P R119)

Sir There are two scenarios; it can be in **term of dollar value** and **number of services**.

We will call to insurance. We will ask.

We will take **denial date** and **claim number**.

What is the **maximum benefit limit?** or **term of service**.

There are two seniors sir

1- If it is **Exceeded in term of amount or number of services**.

We will bill the claim to any other active insurance or otherwise bill to **patient**.

- 2- If it is **not Exceeded**, We will call to the insurance reprocess the claim. We will take a turnaround time and call Reference number

3. **INCLUSIVE/BUNDLED (CO97)**

When the payment of one CPT is included in the payment of other CPT we will get such kind of denials.

We will check on **CCI Edit or Incoder pro** (software) whether CPT are **inclusive or not**

1ST - If found CPT is inclusive, we will send the claim for **coding review**.

2ND -If found CPT is not inclusive, WE WILL to call to the insurance reprocess the claim. We will take a **turnaround time** and **call Reference number**

There two modifiers are used

1. **59-Distinct Procedure Service**
2. **25-for E&M Service**

Note: If the CPT **are not inclusive**, we can also appeal with medical Record, Screen Shot of CCI Edit and EOB.

Note: Modifier 25 is used when two separate evaluation and management service performed on same day separately.

4-GLOBAL (CO97) It includes all the expenses of surgery, pre and post evaluation and management service related to surgery under **global period**:

Global period is of two types for **Major Surgery** it is **90** days and for **Minor surgery** it is **10** days.

We will call to insurance and take **Denial Date** and **Claim Number**.

We will ask what ☐ **Date of Surgery**

What is the **global period**

1st Condition If DOS falls under the global period we will send the claim for coding review.

2nd Condition If DOS falls after the global period.

We will ask the representative to reprocess the claim.

We will take a **turnaround time** and call Reference.

There One modifier used

1. **24** for Unrelated services
2. This modifier is used when patient come for Evaluation and Management Service under global period which is not related to surgery.

5- COVERAGE EXHAUSTED (PR26)

When the patient does not have coverage on **the date of service** Insurance deny the claim for coverage Exhausted.

We will check **eligibility** on the **website**

1st Condition

If found insurance **active** on **the date of service**

WE WILL to call to the insurance reprocess the claim. We will take a **turnaround time** and **call Reference number**

2nd Condition

If found the insurance **not active on the date of service**

We will bill the claim to any other active insurance or otherwise bill to patient.

Note: If patient is above 65 years patient will **have Medicare**. We also need to check whether patient have Medicaid or not.

6- COVERED BY ANOTHER PAYER (C022)

When other insurance is primary on the date of service, we will get such kind of denial.

We will check eligibility on the **website** whether the insurance is **primary or not**.

1st Condition

If found the **insurance is Primary** than we will call to insurance and inform the representative that they are primary for **DOS** and ask the representative to reprocess the claim.

We will take a turnaround time and call Reference.

2nd Condition

- ☐ If found the **other insurance is Primary** than we will update correct insurance as Primary and bill to Insurance.

If we found conflicts between two insurances than we will send a **statement** to the patient for COB update.

7- DUPLICATE C018

If the same service is billed twice or the same service is performed twice and billed without modifier insurance deny the claim for Duplicate.

We need to check in our system whether we have billed the same claim twice or not

1- If we found the same service is Billed twice, we will write-off the duplicate claim.

2- If we found the same service is performed twice and billed without modifier, we will send the claim to coding

There are Two modifier used 76

Modifier -76 if the same service is performed by the same doctor on the same day.

Modifier -77 if the same service is performed by Different doctors on the same day.

If we have billed with modifiers and still claim denied for duplicate, we will call to Insurance and ask the representative to reprocess the claim.

8- The time limit for filing has expired (C029)

Insurance will deny the claim with Denial code CO 29 – The time limit for filing has expired, whenever the claims submitted after the time frame.

- ☐ We will call to Insurance.
- ☐ We will take **Denial Date** and **Claim Number**.
- ☐ **When did they received the claim?**
- ☐ **What is their Timely Filing Limit?**

1st Condition

If Insurance Received **Claim under Timely Filing Limit**. . ☐

- ☐ THAN call to insurance company reprocess the claim take the call referral number

2nd Condition

If insurance received **claim after Timely Filing Limit**.

- ☐ **We will check our billing software** when did we filled the
 - ☐ claim.
 - ☐ If we filled claim after Timely Filing Limit.
- We will write-off the claim.** Yes sir

If we filled claim under Timely Filling Limit.

- ☐ **We will take appeal Limit**
 - ☐ **and appeal address.** Call
- referee number

Action

We will appeal with timely filing Limit Proof.(i.e. Eob and Clearing House Screenshot)

9- No Authorization (CO197)

For every expensive treatment Doctor need to take prior authorization from **insurance company** and that authorization number need to be billed on CMS 1500 form in BOX#23.

We will check our **billing software** whether we have authorization number available or not.

1st Condition

- ☐ If we found **authorization number available** in our system. .
- ☐ We will call to insurance provide authorization number to representative and ask to send claim for reprocess.
- ☐ We will take turnaround time & Call Reference Number.

2nd Condition

If we don't have authorization number available in our billing software.

We will verify place of Service (POS) 23

POS: Location where service was provided.

If place of service is **23 (Which is for emergency)** We don't require authorization number in emergency, so we will ask representative to send the claim for reprocess.

- ☐ However, some insurance want provider to take authorization number even in case of emergency after the service is being performed. So authorization post service is taken **from retro authorization department.**

If place of service is 21 (**Which is for In-Hospital patient**)

- ☐ We will ask representative to send the claim for reprocess with authorization number available with hospital claim.

If they don't have hospital claim or authorization number not available on hospital claim.

We will try to take authorization number **from retro authorization** department.
Hospital claim

If we get authorization from retro authorization Dept. we will ask representative to send claim for reprocess. If we don't get authorization then we **will work as per client protocol.**

10- Claim Not on File

This means claim is not received by insurance company.

We will verify

What is **Mailing address** and **Payer ID**

What is Effective and termination date of policy.

What is Timling Filling Limit of
Insurance.

THERE ARE TWO SENORIO

1-If mailing address or payer id is incorrect, we will **correct** it and we will rebill claim to insurance.

2-If mailing address or payer id is correct, we will look for **clearing .house rejection** and resole it and again rebill claim to insurance.

11- Claim is paid

Claim is paid means claim has been processed by Insurance Company. However, the payment is not posted in system.

So we need to verify

- ☐ Claim Processed Date and Claim Number?
- ☐ What is **allowed amount**, **paid amount** and **patient responsibility**?

What is mode of payment? Like EFT or Check.

If it is EFT we will conform EFT Number and EFT date and in **case of check** we will verify check number and check date.

We will verify payment address. If it is wrong then we will ask the representative to stop the payment and reissue the check on correct address.

Where are check correct provider address

We will check in W9 form